

HOW TO OPEN A BRAIN INJURY TRANSITIONAL LIVING FACILITY in Florida

An AHCA-licensed TLF for adults with brain & spinal-cord injuries — rehabilitation and community re-entry.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact & fee should be re-verified before you rely on it.

The Florida model — read this first

Florida has a purpose-built license for exactly this home: the Transitional Living Facility (TLF), licensed by the Agency for Health Care Administration (AHCA) under Chapter 400, Part XI, Florida Statutes (ss. 400.997–400.9985) and Rule Chapter 59A-17, Florida Administrative Code. A TLF helps adults with moderate-to-severe brain and spinal-cord injuries rebuild independent-living skills and return to the community. It is time-limited, goal-directed rehabilitation — not indefinite custodial care — and it is its own license category (not an assisted living facility, not a nursing home). There is one licensing path (get licensed by AHCA), a funding picture built from BSCIP, Medicaid managed care, and accident insurance, and — this is not optional — a requirement to become nationally accredited within 12 months.

Accreditation is the backbone — put it first. A Florida TLF must attain and continuously maintain accreditation by a rehabilitation-specializing accreditor (CARF or The Joint Commission) within 12 months of its initial license (s. 400.9972). Beyond the legal mandate, accreditation is what wins Medicaid-plan contracts, BSCIP trust, and referrals. This guide treats it as a founding step, not an afterthought — and every PolicyReady product is built to support that accreditation.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written to be understood by someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break that thing down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; requirements and fees change, so verify each step with AHCA and the sources named in Part 11 before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, accredited, funded TLF. Each part below is one rung.

1	Decide this is your business	A licensed, 24-hour rehab facility — capital-intensive but durable (Part 1)
2	Form the business & insure	Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)
3	Secure & prepare the home	Zoning, the Florida Building Code, fire safety, the emergency plan (Parts 3–4)
4	Build & train the team	Administrator, direct-care staff, therapy team — each role's requirements (Part 5)
5	Write your policies	The P&P Manual & Forms Packet — required with the application (Parts 4–6)
6	Get the AHCA TLF license	Apply on the portal; pass the survey; 2-year license (Part 6)

7	Run the program; ACCREDIT ≤12 mo	72-hr eval, 4-day plan; CARF/Joint Commission required (Parts 7, 9)
8	Line up funding	BSCIP, Medicaid SMMC LTC (T2031), auto/PIP, private pay, DVR (Part 8)
9	Get your first clients	BSCIP Central Registry + trauma-center discharge planners (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Do the cheap, reversible steps first (decide, form the entity, lease). Start your accreditation timeline the day you're licensed — CARF requires you to be operating and serving clients for about six months before it will survey you, and you only have 12 months to be accredited.

Before you begin — a realistic picture

Be clear-eyed about what you're taking on. A TLF is a 24-hour licensed health-care facility with a clinical team, a hard accreditation deadline, and vulnerable clients whose safety you're legally responsible for — not a group home you can run casually. Expect a multi-month runway before your first dollar of revenue: several months to form the business, prepare and inspect the building, hire and screen staff, and clear the AHCA survey, then a further push to reach accreditation within 12 months of licensure. Plan for a real capital cushion — the AHCA fee alone is roughly \$5,000+ per biennium, insurance and clinical staffing are ongoing, and you'll carry payroll and rent before your census (and your funding) fills. The upside is durability: Florida is one of the few states with a license built for exactly this work, demand from trauma centers and BSCIP is steady, and a well-run TLF becomes a hard-to-replace community asset. If you go in expecting a licensed clinical operation rather than a quick launch, the path below is entirely doable.

Here's the roadmap. Parts 1–2 explain what this business actually is and who regulates you (AHCA, BSCIP, DCF, the Ombudsman). Parts 3–4 are the hands-on build — forming the entity, insurance, the home, zoning, fire safety, and the emergency plan — broken down step by step. Part 5 covers staffing and required training by position; Part 6 walks you through the AHCA license application, fee, and Level 2 screening. Part 7 traces a client from referral to discharge under Florida's legal timeline; Part 8 shows how you actually get paid; Part 9 lays out costs, timeline, and the accreditation clock. Parts 10–11 get you your first clients and gather every contact and “verify” into one master hub. Read it once through, then work it as a checklist.

PART 1

What This Business Is

In short: A TLF is a licensed, rehabilitation-focused home that helps brain/spinal-injury adults regain independence — its own license category, not an ALF or nursing home. Florida law is strict about who you can admit.

A Florida TLF is a residential facility that assists persons with spinal-cord and head (brain) injuries to achieve a higher level of independent functioning in daily-living skills. It provides rehabilitative services, community re-entry training, aids for independent living, and counseling. Note: Chapter 400 Part XI calls the people you serve “clients,” not residents.

The line that decides everything. A TLF provides transitional rehabilitation and housing. It is not a nursing home or a psychiatric facility. If your model drifts into indefinite custodial care, or into acute medical/psychiatric treatment, you're in the wrong license. Keep it transitional, goal-directed, and community-re-entry-focused.

Who you can admit — and who you can't (s. 400.9973)

Getting this right protects your license and your survey — admitting outside these criteria is a serious, common deficiency.

- **Who qualifies** — a person with an acquired skull, brain, or spinal-cord injury with altered consciousness, OR a spinal-cord lesion / cauda equina syndrome, who shows significant involvement of at least two of these: motor, sensory, cognitive, behavioral, or bowel/bladder function.
- **Who is excluded** — a person whose primary diagnosis is mental illness, an intellectual or developmental disability, or a communicable disease, or who is bedridden, may not be admitted.

Key terms (you'll see these all guide long). TLF = Transitional Living Facility. AHCA = Agency for Health Care Administration (your licensor). BSCIP = Brain & Spinal Cord Injury Program (a funder + the statewide referral registry). SMMC LTC = Statewide Medicaid Managed Care, Long-Term Care. MCO = Managed Care Organization (a Medicaid plan). NFLOC = Nursing Facility Level of Care. CEMP = Comprehensive Emergency Management Plan. NPI = National Provider Identifier. Level 2 = fingerprint background screening. CARF / TJC = the two rehabilitation accreditors.

PART 2

Who Regulates You (and What Each One Does)

In short: *Four bodies touch your TLF: AHCA licenses you, BSCIP funds and refers, DCF protects clients, and the Long-Term Care Ombudsman advocates for them. Know each one's job and how to reach it.*

AHCA — Agency for Health Care Administration

Your licenser. AHCA's Long Term Care Services Unit issues your TLF license, writes the Rule 59A-17 standards, processes your application (AHCA Form 3110-9001) through its online portal, runs the Level 2 background-screening Clearinghouse, and conducts your licensure survey. You'll deal with AHCA more than anyone. Web: ahca.myflorida.com.

BSCIP — Brain & Spinal Cord Injury Program (Florida Dept. of Health)

Two jobs: (1) it FUNDS transitional-living and rehab for eligible clients as the “payer of last resort,” and (2) it runs the Central Registry — the statewide intake that, by law (s. 381.74), receives every new moderate-to-severe brain/spinal-cord-injury case in Florida. That registry is where your referral pipeline begins. Phone 850-245-4045 / 800-342-0778; email DEPCS.BSCIP@FLHealth.gov.

DCF — Department of Children and Families

DCF runs the Florida Abuse Hotline (1-800-962-2873) under Chapter 415, F.S. Your staff are mandatory reporters — if anyone suspects abuse, neglect, or exploitation of a client, they must call that hotline. DCF isn't your licenser, but its abuse-reporting duty runs through your whole operation, and your policies must build it in.

Long-Term Care Ombudsman Program

Independent, volunteer advocates who investigate residents'-rights complaints in long-term-care settings. They can show up in response to a client or family complaint, so treat them as a partner, not an adversary — post their number for clients. Statewide: 1-888-831-0404.

There's no separate “application” for BSCIP/DCF/Ombudsman. Your one licensing application is with AHCA (Part 6). BSCIP involvement comes through a provider agreement + the Central Registry (Part 8 & 10); DCF and the Ombudsman are ongoing duties, not applications. Build relationships with all of them early.

PART 3

Form Your Business & Secure the Home — Step by Step

In short: *This is the part people get stuck on. We break each piece down: what it is, where to go, and exactly what to do.*

3.1 Form your legal entity

An “entity” is the legal business (usually an LLC — Limited Liability Company — or a nonprofit corporation) that will hold the license and the bank account, and that shields your personal assets. You create it with the State of Florida.

- 1. Go to Sunbiz.** The Florida Division of Corporations website is dos.fl.gov/sunbiz. This is the state's official business-registration site.
- 2. File your formation document.** For an LLC, file “Articles of Organization”; for a nonprofit, “Articles of Incorporation.” You can file online. You'll name the business, list a registered agent (a person/address that receives legal mail — can be you), and pay the state fee.
- 3. Save your documents.** Download the filed Articles and your entity's document number — the bank and AHCA will ask for them.
- 4. Consider nonprofit + a board** if you plan to pursue grants or a BSCIP partnership; for-profit LLC is simplest to start.

Stay “in good standing.” Every year you must file an Annual Report on Sunbiz to keep the entity “active.” It's due by MAY 1 (LLC fee \$138.75; for-profit corporation \$150.00 — verify). Miss it and a \$400 late fee hits (for-profits), and eventually the state dissolves your company. Put May 1 on your calendar. Division of Corporations phone: ~850-245-6052 (verify on the Sunbiz contact page).

3.2 Get your EIN (federal tax ID)

An EIN (Employer Identification Number) is your business's Social Security number — a free 9-digit number from the IRS that you need to open a bank account, hire staff, and enroll with payers.

- 1. Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
- 2. Apply online** Monday–Friday during IRS hours. You'll need the responsible party's name and SSN/ITIN and your entity info from Sunbiz.
- 3. Get your EIN immediately** at the end — download and save the confirmation letter (CP-575).

3.3 Open a business bank account

Keep every dollar of the business separate from your personal money — it protects your liability shield and your license. Here's what to bring and where to go.

What to have ready (banks ask for these):

- Your EIN confirmation from the IRS
- Your Sunbiz Articles (showing “active” status) and, if used, your fictitious-name (DBA) certificate
- Operating Agreement (LLC) or By-Laws (corporation)
- A Beneficial Ownership form (identifies anyone owning 25%+ and a control person)
- Government photo ID, SSN, DOB, and home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: any major bank with lots of Florida branches — Chase, Bank of America, Wells Fargo, Truist, PNC, or Regions — or a Florida community bank / credit union that's often lower-fee and more relationship-friendly for a first-time operator, such as MIDFLORIDA Credit Union, Florida Central Credit Union, or Seacoast Bank. Compare: monthly fee and how to waive it, free transaction and cash-deposit limits, branch/ATM access near you, online bill pay, and whether they offer a business line of credit for growth.

3.4 Get your insurance — what you need, who sells it, and what to ask

This is where people get most confused. A TLF cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy. Here is each type in plain English:

Coverage	What it protects against	Must-have?
General Liability (GL)	A visitor/family slip-and-fall or property damage on your premises	Yes — foundational
Professional Liability (malpractice / E&O)	Claims about the CARE itself — med errors, therapy, supervision, neglect	Yes — top healthcare coverage
Abuse & Molestation	Allegations of abuse by staff or others (GL excludes this!)	Yes — high exposure
Commercial Property (+ windstorm/flood)	The building, contents, therapy & medical equipment; add Business Interruption	Yes if you own; often if you lease
Commercial Auto (incl. hired/non-owned)	Transporting clients to therapy/appointments; staff driving own cars for work	Yes if you drive clients
Workers' Compensation	Staff injuries (lifting/transferring is high-risk)	Required in FL at 4+ employees (verify)
Cyber Liability	A breach of protected health information (you're HIPAA-covered)	Strongly recommended
D&O / EPLI	Management decisions; staff employment claims (discrimination, wrongful term.)	Recommended, esp. with a board

Real places to get it

You usually buy through an independent agent/broker who accesses specialty carriers. Carriers that write senior-care / residential-care risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Crum & Forster, Selective, and Nationwide. Specialty programs and brokers that serve exactly this niche include Caitlin Morgan Insurance Services, Gallagher, AssuredPartners, Professional Insurance Group, and XInsurance (for hard-to-place risks). To find a local Florida agent who writes it, use the “Trusted Choice” Find-an-Agent directory at [trustedchoice.com](https://www.trustedchoice.com) (filter healthcare/senior-care), or ask a CARF-accredited peer who they use. Verify each carrier writes your specific TLF class before relying on a quote.

What to ask the insurance agent

“Do you write residential rehabilitation / brain- and spinal-cord-injury facilities specifically — which carriers?”

“Is professional liability included — claims-made or occurrence — and what are the limits?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or full limit?”

“How is Florida windstorm/hurricane and flood handled on the property policy?”

“Are GL and professional liability shared limits or separate?”

“What limits does AHCA (or my landlord/lender) require, and can you meet them?”

“Is the carrier admitted or surplus lines in Florida? What deductible/retention on each line?”

3.5 Secure & zone the home

Choose an accessible house or small building in a real neighborhood near hospitals, therapy, transit, and the community. Before you sign, confirm the location works:

- ☐ Call your city/county PLANNING & ZONING office: is a residential rehabilitation facility for [n] clients an allowed use at this address, or do you need a special/conditional-use permit?
- ☐ Ask whether Florida's community-residential-home siting law (Chapter 419, F.S.) applies and about any spacing/occupancy rules — get the answer in writing.
- ☐ Confirm the building can be made accessible (ramps, doorways, bathrooms) for clients with mobility, sensory, or cognitive impairments.
- ☐ If leasing, make sure the lease allows your use and client count.

On specific room sizes and counts. Florida's TLF rule (59A-17.119) no longer lists square-footage or fixture numbers — the real construction and accessibility specs now come from the Florida Building Code, reviewed by AHCA's Office

of Plans & Construction and your local building department. So the honest answer is: design to the Florida Building Code for your occupancy, and confirm the specifics with AHCA Plans & Construction before you build or renovate (see Part 4).

3.6 Map ownership

Identify every owner and “controlling interest” (anyone with 5%+ ownership or control). You disclose them on the AHCA application, and they — plus all staff with client contact — must clear Level 2 background screening (Part 6).

3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID number that payers use to identify you. Your FACILITY needs a Type 2 (Organizational) NPI. Here's exactly how:

1. **Create an I&A account.** Go to nppes.cms.hhs.gov and register the person who'll manage the number (the “Authorized Official” — usually an owner) in the Identity & Access (I&A) system. Those credentials log you into NPPEs.
2. **Log into NPPEs** at nppes.cms.hhs.gov and start a new application.
3. **Choose Type 2 (Organizational).** Enter the legal business name, your EIN (Type 2 uses the EIN, not an SSN), the business address, and the Authorized Official.
4. **Pick a taxonomy code** (the code describing a residential rehab/transitional-living facility). Confirm the exact taxonomy each payer wants — Medicaid and the MCOs match your enrollment to it.
5. **Submit — it's free.** Most applications process within about 10 business days. Save your NPI.

3.8 Plan your billing (how money will actually flow)

“Billing” means getting paid by each funder. You can't bill until you're set up with each one, so plan the sequence now (the full how-to is Part 8):

1. **Get the NPI (above).** Nothing bills without it.
2. **Enroll as a Florida Medicaid provider** on the FL MMIS Web Portal after running AHCA's Enrollment Readiness Tool (Part 8.2).
3. **Contract with each Medicaid managed-care plan** (Sunshine, Humana, Simply, Florida Community Care, Community Care Plan) — enrollment alone doesn't pay you; the plan contract does (Part 8.3).
4. **Set up a BSCIP provider agreement** and get into the Central Registry flow (Part 8.4).
5. **Decide your private-pay rate and agreement** and how you'll verify auto/PIP or workers'-comp coverage for accident clients (Part 8.5–8.6).
6. **Pick billing help.** Most new facilities use a billing service or clearinghouse to submit claims (often on a UB-04/837I) — budget for it.

PART 4

The Home — Physical Plant, Fire Safety & the Emergency Plan

In short: Design to the Florida Building Code and Fire Prevention Code, keep water safe, plan for power loss, and document everything. Here's what each really means.

4.1 The physical plant — where the real rules live

Rule 59A-17.119 now just says the TLF must have written policies to keep the physical plant safe for clients — it no longer lists room sizes or fixture counts. The actual, enforceable building specs come from the Florida Building Code (for your occupancy classification and accessibility) and are reviewed by AHCA's Office of Plans & Construction plus your local building department. Practically, a compliant TLF has:

- ☐ Accessible bedrooms with, per client, a bed, clean linens/blankets, a place to store clothing and belongings, and adequate space and lighting
- ☐ Enough bathrooms (toilets, sinks, accessible showers) for your client count — sized to the Building Code, not a rule number
- ☐ Dining space, and common + therapy/rehabilitation space set up for independent-living-skills training (a practice kitchen, laundry, and daily-living-skills areas are the point of a TLF)
- ☐ Accessibility throughout (ramps, grab bars, doorway widths, roll-in showers as needed)
- ☐ A clean, safe, well-maintained building with working utilities

Do this before you build or renovate. Confirm your occupancy classification and the exact construction/accessibility specs with AHCA's Office of Plans & Construction and your local building department. That's who gives you the real numbers for your specific building.

4.2 Making common & therapy space “suited to independent-living-skills training”

A TLF isn't just housing — the space itself is a rehab tool. To show a surveyor it's “suited” to the program, set up and document: a training kitchen where clients practice meal prep; a laundry area they use with cueing; a mock or real budgeting/computer station; accessible bathrooms for self-care training; and quiet therapy rooms for PT/OT/speech and behavioral sessions. The rule is met by what the rooms let clients practice, so map each space to a daily-living skill.

4.3 Fire safety

Florida follows the Florida Fire Prevention Code, which adopts NFPA 1 (Fire Code) and NFPA 101 (Life Safety Code). Your LOCAL fire marshal inspects and enforces it, and AHCA also checks life safety at the survey. Required systems generally include smoke detection and alarms, a manual fire-alarm system, portable fire extinguishers, and automatic sprinklers depending on your building's occupancy classification.

The sprinkler question — ask early. Whether full sprinklers are required depends on your NFPA 101 occupancy class (residential board-and-care vs. a health-care/limited-care classification if clients can't self-evacuate). That single classification drives major cost, so confirm it with your local fire marshal and AHCA Plans & Construction before you design. Then get the fire inspection done and keep the signed report.

4.4 Safe hot-water temperature

Scald prevention matters for clients with sensory or cognitive impairment. The TLF rule doesn't state a number; Florida's comparable residential standard (for ALFs/community residential facilities, DOH Rule 64E-12.003) caps resident-accessible hot water at not more than 120°F, and the Building Code's anti-scald plumbing provisions point the same way. A defensible design target is ≤120°F at client-accessible fixtures — confirm the figure for your facility with AHCA.

4.5 The Comprehensive Emergency Management Plan (CEMP) & power loss

A CEMP is your written plan for hurricanes, evacuations, and utility failures, typically filed with your county emergency management agency. It covers who does what, where clients go, how you keep medications and records safe, and how you keep clients safe if the power fails.

***About generators and the 81°F rule.** Florida's post-Hurricane-Irma rule requiring an alternate power source and fuel to keep resident areas at or below 81°F for at least 96 hours is written for nursing homes and assisted living facilities (Rules 59A-4.1265 / 59A-36.025). It is NOT clearly imposed on TLFs by the TLF chapter — but it's the state's safe-temperature benchmark, and it's smart planning. Bottom line: plan for safe indoor temperature during an outage (a properly sized generator is the usual answer), and confirm your exact CEMP and emergency-power obligations directly with AHCA and your county before you build.*

4.6 A few operating rules worth building in now

- **Dietary (Rule 59A-17.118):** meals follow the USDA Dietary Guidelines; a licensed dietitian reviews your menus every year; no more than 14 hours between the evening meal and the next morning meal; plan menus at least a week ahead and keep them six months.
- **Medication by unlicensed staff (Rule 59A-17.125):** untrained/unlicensed staff may give only prepackaged oral, topical, nasal, and eye medications (plus OTCs) — never injections, rectal/vaginal meds, or glucose checks — with signed consent, a triple-check against the label and the record, a 20-minute observation after a new med, and RN supervision.
- **Documented inspections:** keep every fire, building, and health/sanitation inspection report in a single, dated compliance binder (paper and a backup scan) — surveyors will ask for them.

PART 5

Staff, the Rehabilitation Team & Required Training — by Position

In short: Each role has its own qualifications, its own required training and where to get it, and its own screening. Here they are, one by one.

Administrator

- **Role:** runs the facility — operations, staff, and client services — with a designated backup for absences (s. 400.9975).
- **Qualifications:** a qualified administrator responsible for the whole operation; Part XI does not impose the same state “licensed administrator” exam that nursing homes require, but AHCA expects demonstrated competence to run a rehab facility — verify current administrator requirements with AHCA.
- **Training / where:** train on Chapter 400 Part XI / 59A-17, client rights, abuse reporting (DCF hotline), emergency management, and (for accreditation) CARF or Joint Commission standards. AHCA and the accreditors publish guidance; industry associations (e.g., the Florida Association of Recovery/rehab providers, BIAF) offer training.
- **Screening:** Level 2 (Part 6) — required as a controlling interest/administrator.

Unlicensed Direct-Care (UDC) staff

- **Role:** day-to-day care, cueing, supervision, and independent-living-skills support.
- **Required training & validation (Rule 59A-17.126):** if they'll help with medications, they must complete medication-administration training covering 10 specified topics, pass a knowledge test at 100% (up to 3 attempts, or repeat the training), and be VALIDATED by an RN, physician, or pharmacist. Validation is documented (name, license #, date) and expires after 365 days — so re-validate yearly.
- **Where to get it:** from an RN/physician/pharmacist you employ or contract, or an approved medication-administration training provider; keep the certificates.
- **Also train on:** brain-injury competencies — cognitive/behavioral effects, cueing and compensatory strategies, fall/safety management, de-escalation — plus CPR/first aid.
- **Screening:** Level 2 before any client contact.

The interdisciplinary rehabilitation team

Deliver services through licensed professionals suited to your clients — employed or contracted:

- **Physical, Occupational & Speech-Language therapists (PT/OT/SLP):** licensed by the Florida Department of Health; verify each license and keep copies.
- **Rehabilitation nurse (RN):** for health oversight, medication supervision, and staff validation.
- **Neuropsychology / behavioral services:** for cognitive and behavioral supports — licensed professionals.
- **Case management / social work:** coordinates the plan, discharge, funding, and family communication.
- **Medical director:** completes the 72-hour admission evaluation (Part 7) and oversees clinical care.

Document competency, not attendance. For every role: verify the license/credential, run Level 2, complete role-specific training, and keep dated proof of competency in the personnel file. Hold and document regular team meetings on each client's progress.

PART 6

Get Your TLF License from AHCA — the Application, Step by Step

In short: Apply on AHCA's portal with the right attachments and the fee, clear Level 2 screening, and pass the survey. Here's each step, what it asks, and what to have ready.

6.1 Start the application

You apply through AHCA's online licensing portal (the AHCA Single Sign-On Portal at apps.ahca.myflorida.com/SingleSignOnPortal) using the Transitional Living Facility application, AHCA Form 3110-9001, filed under Chapter 408, Part II (the general health-care licensing law) and Chapter 400, Part XI (the TLF law). Chapter 408 Part II is the umbrella process every AHCA facility uses; Chapter 400 Part XI is the TLF-specific content.

The application asks for (have these handy):

- Your entity and every controlling interest / owner (names, addresses, %)
- The facility name, address, and bed capacity
- Proof of liability insurance
- Local zoning compliance signed by a government official, and Chapter 419 compliance
- A satisfactory fire-safety inspection
- A satisfactory county health-department sanitation inspection
- Your Comprehensive Emergency Management Plan (CEMP)
- Your complete Chapter 400 Part XI / 59A-17 policies and procedures (your P&P Manual)
- Background-screening results for controlling interests

6.2 Assemble the required attachments — checklist

- ☐ Entity + controlling-interest / ownership disclosures
- ☐ Proof of liability insurance
- ☐ Zoning compliance (signed) + Chapter 419 compliance
- ☐ Satisfactory fire-safety inspection report
- ☐ Satisfactory county health / sanitation inspection
- ☐ Comprehensive Emergency Management Plan
- ☐ Chapter 400 Part XI / 59A-17 policies & procedures (P&P Manual) and referenced forms
- ☐ Level 2 screening initiated for all controlling interests

Where your manual & forms fit. The application requires complete Part XI / 59A-17 policies and procedures. PolicyReady's TLF Policy & Procedure Manual and Forms Packet are built to that requirement and to the accreditation frameworks — have them customized and ready before you apply.

6.3 Pay the fee (budget for this — it's hefty)

The TLF fee is a \$4,588 license fee PLUS a \$90 per-bed fee, per biennium (a two-year license) — so a 10-bed home is roughly $\$4,588 + \$900 \approx \$5,488$ every two years. The base amounts can be adjusted annually (s. 408.805), so confirm the current number on AHCA's fee schedule and the statute (s. 400.9972). Fees are non-refundable.

What “a late application triggers a \$50/day penalty” means. “Late” means you didn't file on time — for a renewal, you must submit before your current license expires; for a change that requires a new/updated license, you must file within the required window. Miss it and AHCA adds \$50 for every day you're late, on top of the fee. Translation: file your renewal early — set a reminder ~90 days before expiration.

6.4 Clear Level 2 background screening

Every controlling interest and every staff member with client contact must CLEAR Level 2 screening before client contact. Here's what that involves:

- **What it is:** fingerprint-based state (FDLE) and national (FBI) criminal-history check, submitted by Livescan and processed through AHCA's Care Provider Background Screening Clearinghouse.
- **What can disqualify:** the offenses listed in s. 435.04 plus the extra ones in s. 408.809(4) — e.g., Medicaid fraud, abuse/battery of a vulnerable adult, patient brokering, identity theft. Not every record disqualifies; many people come back “Eligible.”
- **Re-screening:** every 5 years.
- **If disqualified:** the person can apply to AHCA for an exemption under s. 435.07 by proving rehabilitation (clear and convincing evidence), but can't work in the role until the exemption is granted.

6.5 Pass the licensure survey — a pre-survey checklist

After AHCA accepts your application it schedules a survey — an on-site review against Chapter 400 Part XI and Rule 59A-17. A new-facility (initial) survey is often announced, but plan as if a surveyor could walk in and ask for anything below. Have it all assembled, current, and in one place before the visit:

- ☐ Your active AHCA license application and the paid fee receipt
- ☐ The complete P&P Manual and Forms Packet, matched section-by-section to 59A-17
- ☐ Signed, satisfactory fire-safety inspection and county health/sanitation inspection reports
- ☐ The Comprehensive Emergency Management Plan and county filing confirmation
- ☐ Level 2 “Eligible” results for every controlling interest and every staff member with client contact
- ☐ Personnel files: license/credential verification, training certificates, med-admin validation dates, job descriptions
- ☐ Proof of liability insurance meeting AHCA-required limits, plus the current declarations page
- ☐ Signed zoning-compliance letter and Chapter 419 confirmation
- ☐ Blank client-record templates showing you can capture the 72-hour eval, 4-day plan, and monthly reviews

What this means for you. A surveyor is checking that your paper promises match reality — that the policy exists, the staff are screened and trained, the building is safe, and the client record can be produced on demand. If a document is missing or a date has lapsed, that's a deficiency you'll have to correct on a Plan of Correction. Do a full dry run a week before: pull every item above, verify each date is current, and fix gaps while you still have time.

PART 7

The Client's Journey — Referral to Discharge, Broken Down

In short: Florida law sets the clock: eval in 72 hours, a treatment plan in 4 days, monthly reviews, 30 days' notice to discharge. Here is the whole journey, stage by stage.

Quick-reference timeline (the full breakdown follows):

When	What must happen	Citation
At/before admission	Confirm admission criteria; begin a discharge plan naming the target setting & skills needed	s. 400.9973
Within 72 hours	Medical director completes the initial evaluation	s. 400.9973
Within 4 days	Comprehensive treatment plan implemented	s. 400.9974
At least monthly	Team reviews/updates the discharge plan & progress	s. 400.9973
Evaluation-only cases	No more than 90 days	s. 400.9973
To discharge	≥30 days' notice; discharge when goals met / no progress / less-restrictive option exists	s. 400.9973

Stage 1 — Referral & pre-admission screening

A referral arrives (usually from the BSCIP Central Registry or a hospital discharge planner — Part 10). You screen it against the s. 400.9973 admission criteria (qualifies vs. excluded, Part 1) and verify funding BEFORE admission (Part 8) — never admit without confirming who pays and what they cover.

Stage 2 — Admission

The client (and family) sign the admission/residency agreement and consents, receive an orientation to their rights and the house, and you complete a personal-property inventory (s. 400.9981 protects the client's property and personal affairs). Explain the program, the goals, the fees, and the rules plainly.

Stage 3 — Medical-director evaluation (within 72 hours)

The medical director completes an initial evaluation of the client within the first 72 hours — the medical/functional baseline the treatment plan is built on.

Stage 4 — Interdisciplinary assessments

Each discipline assesses the client: PT (mobility, transfers, falls), OT (daily-living skills, adaptive equipment), SLP (speech, swallowing, cognition), nursing (health, meds), neuropsychology/behavioral (cognition, behavior), and case management/social work (supports, funding, discharge target).

Stage 5 — The comprehensive treatment plan (within 4 days, s. 400.9974)

Within 4 days of admission you implement an individualized comprehensive treatment plan with measurable community-reintegration goals, independent-living-skills training, and neurobehavioral supports. It's built by the interdisciplinary team, individualized to the client, developed WITH the client/family, and signed. This is the spine of the whole stay.

Stage 6 — The daily program

Daily life is the rehab: scheduled PT/OT/speech and behavioral sessions, independent-living-skills training (cooking, laundry, budgeting, medication self-management with cueing), community re-entry outings, and structured routines. A sample day might run morning ADLs and therapies, a midday skills group, afternoon community outing or vocational activity, and evening life-skills practice — all mapped to the plan's goals.

Stage 7 — Monthly progress reviews

At least monthly, the interdisciplinary team meets, measures progress against the goals, updates the treatment and discharge plans, and documents it. Progress (or its absence) drives what happens next.

Stage 8 — Restraint, seclusion & safety limits (s. 400.9979)

Florida strictly limits restraint and seclusion — they are safety measures of last resort, not behavior tools, and must follow your written policy, be documented, and protect client safety and rights. Build de-escalation first; use restraint/seclusion only as the law allows.

Stage 9 — Discharge & transition

The discharge plan started at admission: it names the target setting (independent apartment, family home, supported living) and the skills the client needs to get there, and it's reviewed monthly. Give at least 30 days' notice, and discharge when the client meets the goals, shows no measurable progress, or the TLF is no longer the least-restrictive option. Hand off warmly to the next setting and follow up.

It's transitional, by law. Everything — your program, documentation, and outcomes — should show clients moving toward greater independence and out to the community, not staying indefinitely.

What this means for your records

Every stage above must be documented — the client record is what a surveyor, an MCO auditor, and an accreditor all read to decide whether your program is real. If it isn't written down, it didn't happen. For each client, keep a complete, dated file that traces the whole journey and proves you hit every statutory deadline:

- ☐ Signed admission/residency agreement, consents, client-rights acknowledgment, and personal-property inventory (s. 400.9981)
- ☐ Documented screening against the s. 400.9973 admission criteria and pre-admission funding verification
- ☐ The medical director's initial evaluation, dated within 72 hours of admission
- ☐ The comprehensive treatment plan, implemented within 4 days, with measurable goals and client/family signatures
- ☐ Interdisciplinary assessments (PT, OT, SLP, nursing, neuropsych/behavioral, case management)
- ☐ Monthly interdisciplinary progress reviews with updates to the treatment and discharge plans
- ☐ Medication administration records and any med-admin validation for the staff involved
- ☐ Any restraint/seclusion documentation per s. 400.9979, with the safety justification
- ☐ The discharge summary, the ≥30-day notice, and the warm hand-off to the next setting

A practical habit. *Build a standard client-file order (a tabbed binder or an EHR template) and audit two or three files each month against this list. Catching a missing monthly review or an unsigned plan yourself — before a surveyor or auditor does — keeps small gaps from becoming citations, denied claims, or accreditation findings.*

PART 8

How You Get Paid — Lining Up Funding, Step by Step

In short: *There's no single TLF Medicaid per-diem. You assemble funding from BSCIP, Medicaid managed care, accident insurance, private pay, and Voc Rehab — and you verify each client's funding BEFORE you admit.*

Summary of the sources (each is broken down below):

Source	What it is	Key fact
BSCIP	Florida's dedicated brain/spinal-injury funder + referral registry	Payer of last resort; refer via the Central Registry
Medicaid SMMC Long-Term Care	Managed care — the MCO pays the facility	Bill assisted-living under HCPCS T2031; room & board NOT covered
Auto (Florida PIP) / workers' comp	Accident-related clients	PIP \$10,000 min; EMC needed for the full \$10k
Private pay	Family/self-pay	You set the rate; use a written agreement
Vocational Rehabilitation (DVR)	Work-re-entry services	Authorized through the client's IPE

8.1 How lining up funding actually works

For each client you'll confirm what coverage they have and in what order it pays, then bill accordingly. For the business overall, you get SET UP with each funder once (below), and then per client you verify coverage before admission. Accident clients often stack: PIP first (capped), then MedPay/health insurance/liability settlement, then BSCIP as last resort; Medicaid-eligible clients with a nursing-facility level of care go through an MCO.

8.2 Medicaid — enroll as a provider (do this first)

1. **Run the Enrollment Readiness Tool** at ahca.myflorida.com/provider/readiness.html — it tells you your provider type, required documents, and risk level.
2. **Apply on the Florida MMIS Web Portal** (the state's Medicaid system, run by fiscal agent Gainwell) at portal.flmmis.com. Provide your NPI (Type 2), EIN/W-9, facility license, and ownership disclosures.
3. **Complete Level 2 screening** through the Clearinghouse (Part 6.4).
4. **Get your Medicaid ID.** Enrollment establishes your provider record — but managed-care clients are paid through the plans, so you must also contract with each plan (next). AHCA Provider Services: 1-800-289-7799.

Questions Medicaid will ask you:

- NPI, EIN, licensure, ownership/managing employees, background-screening results, service location, provider type/specialty.

Questions you should ask:

- Which provider type/specialty covers a TLF? What risk category am I (affects fingerprinting/site visit/fee)? Do I need to be enrolled before an MCO will contract me (usually yes)?

8.3 Medicaid managed care — contract with the plans

Under SMMC 3.0 (contracts effective Oct 1, 2024), the current Long-Term Care plans are Sunshine Health, Humana, Simply Healthcare, Florida Community Care, and Community Care Plan (participation is regional — verify your region on AHCA's SMMC pages). Aetna and Molina are NOT current LTC awardees.

1. **Be Medicaid-enrolled first** and hold your Type 2 NPI.
2. **Request a contract** from each plan's Provider/Network Relations department (each has a “Join Our Network” form).
3. **Credential:** submit your license, NPI, W-9/EIN, insurance certificates, accreditation, ownership disclosures, and quality info. Plans re-credential ~every 3 years.
4. **Execute the contract / rate.** A TLF may negotiate a single-case agreement per client if the plan doesn't have a standard TLF rate.

5. Per client: the plan's case manager builds a plan of care, and the client must meet Nursing Facility Level of Care (NFLOC), determined by the state CARES assessment.

Coding & money: assisted-living services bill under HCPCS T2031 ("Assisted living, waiver; per diem"). Room and board is NOT covered — the client pays that from their own income and keeps a monthly personal-needs allowance (about \$130–\$160 — verify current).

Questions the MCO will ask you:

– NPI, Medicaid ID, license, insurance, accreditation, staffing/qualifications, service array, rates, capacity, quality data.

Questions you should ask the MCO:

– Do you cover transitional-living/residential rehab, or is it a single-case agreement? What code and per-diem rate (T2031 or negotiated)? Who authorizes NFLOC and how are auths renewed? How is room-and-board / the client's personal-needs allowance handled? Claim format, timely-filing window, prior-auth rules?

8.4 BSCIP — the dedicated brain/spinal-injury funder

BSCIP funds transitional-living and rehab for eligible clients, but as the PAYER OF LAST RESORT — it makes you bill Medicaid, private insurance, PIP, workers' comp, and any settlement first, and it can place liens on injury settlements (s. 381.79). Its Central Registry is also your referral engine (Part 10).

1. **Contact the BSCIP central office** (850-245-4045 / 800-342-0778) to request the provider/vendor agreement — the vendor-contracting steps aren't posted online, so you call for them.
2. **Sign the provider agreement** and learn the authorized rate and how per-client authorizations are issued.
3. **Take Central Registry referrals** (Part 10) and coordinate with the assigned BSCIP case manager.

Questions BSCIP will ask you:

– Facility license (Rule 64I-1.005 / AHCA TLF license), NPI, services/outcomes, rates, and your agreement to bill other payers first and accept last-payer/lien terms.

Questions you should ask BSCIP:

– What's the vendor-contracting process and authorized rate? What proof that other coverage is exhausted do you need? How are authorizations issued and renewed per client? How do liens/third-party recoveries affect what you're actually paid?

8.5 Auto (Florida PIP) & workers' comp — accident-related clients

- **Florida PIP (no-fault):** every FL auto policy has a \$10,000 PIP minimum and pays 80% of reasonable, necessary medical/rehab costs. The person must get initial care within 14 days of the crash, and a provider must certify an Emergency Medical Condition (EMC) to reach the full \$10,000 — without an EMC the cap drops to \$2,500. \$10k goes fast in residential rehab, so PIP is usually a first, partial payer.
- **Workers' comp:** no dollar cap; can fund long-term rehab for a compensable on-the-job injury — but you need authorization from the comp carrier/adjuster and bill under the FL Workers' Comp reimbursement manual.

Ask the auto/comp adjuster:

– Is there an EMC determination (so the PIP cap is \$10k not \$2,500)? What's the remaining benefit balance? Is MedPay or bodily-injury coverage available? Who authorizes residential level of care, and at what rate/code?

8.6 Private pay & Vocational Rehabilitation

- **Private pay:** you set a daily or monthly rate (room & board + program), with add-ons for higher acuity. Use a written residency/financial agreement covering the rate and what's included, deposit, billing cycle, refund/proration on discharge, rate-change notice, the financial-responsibility guarantor, discharge criteria, and coordination with any secondary payer. Have counsel review it against the licensure rule.
- **Ask a private-pay family:** who is financially responsible; is there PIP, a settlement, or long-term-care insurance to coordinate; and do they understand the rate, what's included, and the refund terms?

- **Division of Vocational Rehabilitation (DVR, rehabworks.org):** funds work-re-entry services. The client must be found eligible (an impairment that's a substantial barrier to employment, needing VR services); SSI/SSDI recipients are presumptively eligible. DVR determines eligibility within 60 days, writes an Individualized Plan for Employment (IPE), and issues written authorizations to vendors — you become a DVR vendor and bill only pre-authorized services.

8.7 A worked example — how a single client's funding stacks

Funding rarely comes from one source. Here's an illustrative (not a quoted rate) walk-through for one accident client, to show how the pieces fit and why you verify coverage before you admit:

- **Day 1 — the crash file:** a 34-year-old is admitted after a car crash with a traumatic brain injury. She has auto insurance with the \$10,000 PIP minimum, got emergency care within 14 days, and a provider certified an Emergency Medical Condition — so the full \$10,000 PIP is available (not the \$2,500 cap).
- **First payer — PIP:** PIP pays 80% of reasonable, necessary rehab costs until the \$10,000 is exhausted. In residential rehab that goes fast — often within the first few weeks — so you treat it as a partial, first-in payer, not the plan for the whole stay.
- **Next — settlement or health coverage:** if she has MedPay, health insurance, or a pending bodily-injury/liability settlement, that's coordinated next. You document what's available and its balance.
- **Backstop — BSCIP:** once earlier payers are exhausted or unavailable, BSCIP can authorize transitional-living funding as payer of last resort — after you've shown other coverage is used up, and subject to its lien on any injury settlement (s. 381.79).
- **Alongside — DVR:** as she stabilizes and work re-entry becomes a goal, DVR can authorize pre-approved vocational services under her IPE, billed separately.

The lesson: verify before you admit. Before this client ever moves in, you confirm the EMC status and PIP balance, identify any settlement or secondary coverage, and open the BSCIP conversation so the last-resort funding is lined up. Admitting first and chasing payment later is how facilities end up delivering months of unfunded care. Every client gets a documented funding plan on the front end.

PART 9

What It Costs & How Long It Takes

In short: Budget the building, the team, the AHCA fee, and accreditation. Plan a multi-month runway — and remember accreditation runs on its own ~12–18-month, staged clock.

9.1 Startup line items (typical ranges — verify)

Item	What's in it	Ballpark
The home	Lease deposit + first month (or purchase); accessibility mods; furnishings; life-safety systems	Varies widely by market/size
Insurance	GL + professional + abuse + property + auto + workers' comp (annual)	Get quotes; a package for a small facility is a real annual line
Clinical staffing / therapy	PT/OT/SLP, nursing, behavioral — employed or contracted	Your largest ongoing cost
Training & screening	Med-admin training/validation, CPR, Level 2 fingerprinting fees	Per person; modest each
AHCA license fee	\$4,588 + \$90/bed per 2-year biennium	~\$5,000+ (10-bed ≈ \$5,488)
Business setup	Sunbiz filing, EIN (free), NPI (free), bank, legal	Low hundreds + legal
Policies	P&P Manual + Forms Packet	One-time
Accreditation	CARF or Joint Commission (see 9.2)	Low-to-mid five figures over the cycle

9.2 What accreditation costs (CARF & Joint Commission)

- **CARF:** roughly a \$995 application fee plus about \$1,525 per surveyor, per day (a small program's survey is ~2–3 days with 1–2 surveyors), on a 3-year cycle with no separate annual fee (you file an annual conformance report). Realistic direct CARF cost for a small single-program facility is low-to-mid five figures — verify by quote. CARF also offers a Brain Injury Specialty designation. Phone (888) 281-6531, carf.org.
- **The Joint Commission:** an annual fee plus a one-time survey fee, both priced from your services and average daily census (a small residential program pays at the low end), on a 3-year cycle. TJC doesn't publish fixed figures — request a quote from its Pricing Unit, 630-792-5115, jointcommission.org.

9.3 Accreditation happens in STAGES — plan for it

Accreditation is not a one-day event. The typical path (CARF, which is the common choice for brain-injury residential rehab):

1. **Self-study / gap analysis** against the current standards — do this before you apply.
2. **Operate and serve clients first.** CARF requires you to be OPEN and serving clients in conformance to the standards for about SIX MONTHS before it will survey — you cannot skip this.
3. **Apply** at least 3 months before your desired survey date.
4. **Initial on-site survey** (~2–3 days): peer surveyors observe, interview staff and clients, and review documents.
5. **Decision in ~6–8 weeks** plus a Quality Improvement Plan. Terms: Three-Year (best), One-Year (probationary — you're re-surveyed the next year), Provisional, or Non-Accreditation.

Reconcile the two clocks. Florida requires accreditation within 12 months of licensure, but CARF wants ~6 months of operation before it surveys and ~6–8 weeks to decide — so realistically you open, immediately begin serving clients and gathering outcome data, apply as early as CARF allows, and drive toward the survey. Total start-to-accreditation is commonly ~12–18 months; start day one and stay in close contact with the accreditor and AHCA about the timeline.

9.4 Timeline

Weeks	Business & home ready	Entity, insurance, accessible building, zoning confirmed
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Weeks	Team & policies ready	Administrator + staff hired, Level 2 + validation, P&P adopted
~60+ days	AHCA application in review	Portal application + attachments + fee; then the survey
Survey	License issued	Pass the AHCA survey → 2-year TLF license
Months 1–12	Serve clients & ACCREDIT	Operate → CARF/JC survey → accreditation (the 12-mo mandate)
Ongoing	Funding & full census	BSCIP + MCO contracts + private pay; keep beds full

9.5 Plan your cash runway — the gap before revenue

The single most common way a new TLF gets into trouble isn't the license fee — it's underestimating how long you'll carry costs before money comes in. Rent, insurance, and the first hires start before your first client; the AHCA survey and license take weeks after your application is complete; then Medicaid enrollment, MCO credentialing (which can run a few months), and BSCIP contracting each add lead time before claims actually pay. Build a runway that assumes you're funding operations for several months at partial census.

- **Pre-license burn:** rent/mortgage, accessibility build-out, insurance premiums, and early payroll all run before you can admit a single client.
- **Ramp-to-census:** beds fill gradually as referrals and authorizations come through — model 3–6 months to reach a healthy occupancy, not week one.
- **Claims lag:** even once you're contracted, expect weeks between delivering care and receiving payment; credentialing and prior authorizations can push the first checks out further.
- **Cushion:** hold enough working capital to cover fixed costs at low census while contracts and referrals mature — a line of credit (Part 3.3) is a useful backstop.

***What this means for you.** Treat funding contracts as parallel work, not a step you do after opening. Start Medicaid enrollment, MCO credentialing, and the BSCIP agreement as early as the rules allow, so the payment pipeline is live by the time your beds fill — otherwise you'll deliver real care for months before real revenue arrives.*

PART 10

Getting Your First Clients — the Referral Pipeline

In short: By law, every new moderate-to-severe brain/spinal-cord injury in Florida is referred to the BSCIP Central Registry — get known there, and to the trauma centers, and your beds fill.

10.1 The rule that builds your pipeline

Florida law (s. 381.74) requires all hospitals, physicians, and agencies to refer every new moderate-to-severe TBI/SCI to the BSCIP Central Registry. A BSCIP case manager then contacts the person (stated as within about 10 business days) to assess eligibility and authorize services — including transitional living. So being a known, contracted BSCIP provider puts you in front of the entire statewide pipeline.

BSCIP Central Registry — how referrals flow:

- Online portal: bscipresourcecenter.org (Central Registry form)
- Fax referrals: 850-410-1975
- BSCIP office: 850-245-4045 / 800-342-0778 • DEPCS.BSCIP@FLHealth.gov

10.2 Trauma centers & rehab hospitals, by metro (verify before relying)

Discharge planners and case managers at these programs place brain/spinal-injury patients who need transitional living. Introduce yourself, leave a one-pager, and ask how to get on their housing-referral list.

Metro	Program (referral source)	Contact (verify)
Miami / S. FL	Ryder Trauma Center — Jackson Memorial (rehab)	305-585-1152 • jacksonhealth.org
Miami / S. FL	Baptist Health Brain & Spine Care	baptisthealth.net (verify #)
Tampa Bay	Tampa General Hospital (rehab / brain injury)	813-844-7000 • tgh.org
Tampa Bay	AdventHealth Tampa — neuro / TBI	adventhealth.com (verify #)
Orlando	Orlando Health Advanced Rehab Institute — Brain Injury	407-407-0500; inpatient 321-841-4120
Jacksonville	UF Health Jacksonville / TraumaOne	904-244-0411 • ufhealthjax.org
Gainesville	UF Health Shands + UF Health Rehab Hospital	rehabhospitals.ufhealth.org (verify #)
Pensacola	Ascension Sacred Heart — Brain & Spine	healthcare.ascension.org (verify #)

10.3 Advocacy & veterans

- **Brain Injury Florida (Brain Injury Association of FL):** statewide helpline 1-800-444-6443, info@braininjuryfl.org, Clearwater — routes families and survivors to resources; a referral and community partner.
- **VA — James A. Haley Veterans' Hospital, Tampa:** a national VA Polytrauma Rehabilitation Center with a transitional rehab program — connect with its social workers for veteran referrals. va.gov/tampa-health-care.

What to ask each referral source:

- How do you decide where to place brain/spinal-injury patients who need transitional living? What do you need from me to be on your referral list (capabilities, accreditation, availability)? Who is the discharge planner / case manager I should stay in touch with?

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: This is your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, screening & the building

Who	Contact	What to verify / ask
AHCA — Long Term Care Services Unit	ahca.myflorida.com • Provider Services 1-800-289-7799	Current TLF fee & fee schedule; the application (Form 3110-9001) & what to attach; the survey; current administrator requirements
AHCA Office of Plans & Construction	via ahca.myflorida.com	Your occupancy classification & exact physical-plant/accessibility specs before you build/renovate
AHCA Background Screening Clearinghouse	ahca.myflorida.com (Clearinghouse)	How to initiate Level 2; disqualifying offenses; the exemption (s.435.07) process
Local Fire Marshal / Building Dept.	your city/county	NFPA 101 occupancy class & whether full sprinklers are required; the fire inspection
City/County Planning & Zoning	your jurisdiction	Allowed use at your address; Ch. 419 siting; occupancy/spacing — in writing
County Health Department	your county	The sanitation inspection; the safe hot-water temperature for your facility
County Emergency Management	your county	Your CEMP filing duty and any emergency-power expectation

Business, insurance & billing IDs

Who	Contact	What to verify / ask
FL Division of Corporations (Sunbiz)	dos.fl.gov/sunbiz • ~850-245-6052	Filing your entity; the May 1 annual report to stay in good standing
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppes.cms.hhs.gov • cms.gov	Your Type 2 NPI and the right taxonomy for a TLF
Insurance agent (Trusted Choice)	trustedchoice.com; carriers/programs in Part 3.4	Which coverages & limits; abuse & molestation; windstorm/flood; AHCA-required limits
Business bank	Chase/BofA/Wells/Truist/PNC/Regions; MIDFLORIDA, Florida Central CU, Seacoast	Account fees, transaction/cash limits, business line of credit

Funding & referrals

Who	Contact	What to verify / ask
BSCIP (DOH)	850-245-4045 / 800-342-0778 • DEPCS.BSCIP@FLHealth.gov • Central Registry fax 850-410-1975	Provider/vendor agreement & rate; last-resort/lien terms; per-client authorizations; Registry referrals
Florida Medicaid (AHCA / MMIS)	Readiness Tool + portal.flmmis.com • Provider Services 1-800-289-7799	Provider type/specialty for a TLF; risk category/fees; enrollment steps
SMMC LTC plans	Sunshine, Humana, Simply, Florida Community Care, Community Care Plan	Coverage/single-case; T2031 rate; NFLOC auth; room-and-board/PNA; claims & timely filing
Auto (PIP) / workers' comp adjuster	the client's carrier	EMC determination; remaining benefits; authorization & rate for residential rehab
DVR (Vocational Rehab)	rehabworks.org	Becoming a vendor; authorizable services; fee schedule; IPE authorizations
Accreditor — CARF / TJC	CARF (888) 281-6531 carf.org • TJC 630-792-5115 jointcommission.org	Exact cost quote; the ~6-month operating requirement & survey timeline; Brain Injury specialty

Advocacy, oversight & referral partners

Who	Contact	Role
Long-Term Care Ombudsman	1-888-831-0404 • ombudsman.elderaffairs.org	Independent client-rights advocate — post for clients
DCF Florida Abuse Hotline	1-800-962-2873	Mandatory abuse/neglect/exploitation reporting
Brain Injury Florida	1-800-444-6443 • braininjuryfl.org	Referrals, family resources, community partner
Trauma centers / rehab hospitals	see Part 10	Your primary discharge-planner referral sources

You can do this

Opening a Florida TLF is a serious build — a licensed building, a trained clinical team, real clients, and a hard accreditation deadline — but it's a defined path, and Florida is one of the few states with a license made for exactly this work. Take it one rung at a time: form the business, prepare the home and team, get licensed by AHCA, serve clients while you drive toward CARF or Joint Commission accreditation, assemble funding, and earn referrals from BSCIP and the trauma centers. Verify each fee and contact as you go — and lean on the people in Part 11; that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in Chapter 400 Part XI, F.S., Rule 59A-17, F.A.C., and current public program information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with AHCA, BSCIP, the accreditor, and qualified counsel before you rely on it. A TLF is transitional and rehabilitation-focused, and Florida has no single brain-injury Medicaid per-diem — confirm each client's funding before admission.